

RUSH UNIVERSITY MEDICAL CENTER
1620 W. Harrison Street · Chicago, IL 60612 · (312) 942-5000
CERTIFIED MEDICAL RECORD — TRAUMA / EMERGENCY SERVICES

Patient Name:	Henderson, James William	MRN:	RUMC-4419823
DOB:	08/14/1982	Age:	42
Sex:	Male	Account:	ACC-202503152291
Admission Date:	03/15/2025	Discharge Date:	03/17/2025
Attending:	Dr. Yolanda Park, MD (ER)	Disposition:	Admitted → Rehab

EMERGENCY DEPARTMENT TRIAGE NOTE — 03/15/2025 09:51

Patient is a 42-year-old male presenting via EMS following a motor vehicle collision (rear-end impact, I-90 eastbound) approximately 34 minutes prior to arrival. Patient complains of severe cervical and lumbar pain (8/10), global headache, dizziness, nausea, and intermittent confusion. EMS reports loss of consciousness was not confirmed but patient was initially disoriented at scene. Vitals on arrival: BP 148/92, HR 104, RR 18, SpO2 99% on room air, GCS 14 (E4V4M6).

ATTENDING PHYSICIAN NOTE — Dr. Yolanda Park, MD — 03/15/2025 11:20

History of Present Illness: Mr. Henderson is a 42-year-old male civil engineer presenting after a high-energy rear-end MVA on I-90. He was a properly restrained driver. The airbag deployed. He denies pre-existing spine or neurological conditions. He reports the following persistent symptoms: severe neck pain radiating to bilateral shoulders (L > R), low back pain with radiation down the left leg to the knee, frontal headache, photophobia, difficulty concentrating, and subjective memory gaps regarding the immediate post-impact period.

Imaging Results — CT Head without contrast (03/15/2025 10:14): No intracranial hemorrhage. No skull fracture. Normal gray-white differentiation. Mild diffuse cerebral edema consistent with concussive mechanism. Impression: Mild traumatic brain injury (mTBI).

MRI Cervical Spine without contrast (03/15/2025 10:52): Straightening of the normal cervical lordosis consistent with muscle spasm. Disc bulge at C5-C6 with mild bilateral foraminal narrowing. No cord signal abnormality. Impression: Grade III cervical strain / sprain with C5-C6 disc bulge.

MRI Lumbar Spine without contrast (03/15/2025 11:05): Herniated nucleus pulposus at L4-L5 with leftward paracentral protrusion contacting and compressing the descending left L5 nerve root. Foraminal stenosis moderate at L4-L5 left. No evidence of pre-existing degenerative changes at this level. Impression: Traumatic left L4-L5 disc herniation with left L5 radiculopathy.

Assessment: (1) Mild traumatic brain injury (ICD-10: S06.0X0A); (2) Herniated intervertebral disc, L4-L5, with left L5 radiculopathy (M51.16); (3) Cervical strain, grade III (S13.4XXA); (4) Acute pain disorder secondary to MVA (F45.41).

Plan: Admit for observation and neurological monitoring. IV ketorolac for pain control. Neurosurgery consult for L4-L5 herniation. Physical therapy evaluation. Refer to Shirley Ryan AbilityLab for inpatient rehabilitation on discharge.

INPATIENT COURSE — 03/15 to 03/17/2025

Patient remained neurologically stable during two-night observation. Headache and confusion showed gradual improvement. Lumbar radiculopathy persisted. Physical therapy evaluation performed on 03/16; PT recommended inpatient rehabilitation. On 03/17, patient transferred to Shirley Ryan AbilityLab (formerly Rehabilitation Institute of Chicago) for inpatient rehabilitation admission under Dr. Amara Singh, PM&R.;

SHIRLEY RYAN ABILITYLAB — INPATIENT REHABILITATION NOTE

Attending Physiatrist: Dr. Amara Singh, MD, FAAPMR
Admission: 03/17/2025 | Discharge: 04/28/2025 (42 days)

Mr. Henderson underwent comprehensive inpatient rehabilitation for six weeks following traumatic L4-L5 disc herniation and mTBI. Program included daily physical therapy (lumbar stabilization, gait training), occupational therapy (cognitive rehabilitation for post-concussive symptoms), neuropsychological evaluation (Dr. Robert Cain, PhD — confirming moderate neurocognitive deficits consistent with mTBI), and pain management consultation. At discharge, patient ambulated independently with a cane, continued to exhibit left-leg weakness (4/5 hip flexion, 4-/5 great toe extension), and reported headaches 4–5 days per week. Discharged to home with outpatient PT 3x/week and referral to Dr. Kevin Shaughnessy (Orthopedic Spine Surgery) for evaluation of surgical intervention.

BILLING SUMMARY — CERTIFIED THROUGH 04/01/2025

Service	Provider	Date(s)	Amount
Emergency Dept — Level V	Rush University Med Ctr	03/15/2025	\$4,812.00
CT Head w/o Contrast	Rush Radiology	03/15/2025	\$1,240.00
MRI Cervical Spine	Rush Radiology	03/15/2025	\$2,380.00
MRI Lumbar Spine	Rush Radiology	03/15/2025	\$2,380.00
Inpatient Observation (2 nights)	Rush University Med Ctr	03/15–03/17	\$11,420.00
Inpatient Rehabilitation (42 days)	Shirley Ryan AbilityLab	03/17–04/28	\$63,420.00
Neuropsychological Evaluation	Shirley Ryan AbilityLab	04/02/2025	\$1,882.62
		TOTAL:	\$87,534.62

This is a certified true copy of the medical record of James William Henderson (MRN RUMC-4419823). Released pursuant to patient authorization dated April 3, 2025. Records Custodian: Doris L. Kim, RHIA, Rush University Medical Center.